1 Table 4 Test

Table 1: Clinical Outcomes

Strategy	Implants	30-Yr Surv	Life-Years	SCD Deaths	SCDs Saved	NNT (SCD)	NNT (All)	LYs / Imp
No ICD (Do Nothing)	0 (0.0%)	49.6%	14.54 yrs	653	—	—	—	—
ESC Risk ¿ 6%	538 (14.7%)	55.2%	15.11 yrs	397	255	2.1	2.6	+3.93
Observed Practice	559 (15.2%)	54.5%	15.01 yrs	421	232	2.4	3.1	+3.11
ESC Risk ¿ 4%	1106 (30.1%)	57.9%	15.38 yrs	275	378	2.9	3.6	+1.91

2 Table 7 Test

Table 2: Threshold Analysis

Decision Context	Break-Even (£25k)	Break-Even (£35k)	Current ICER	Economic Rationale
Head-to-Head Parity (Risk ¿ 6%)	£12,761 (-26.8%)	£12,773 (-26.7%)	Dominated	Must be discounted ˜£2,550 below TV-ICD list price (£15,317) to compensate for 8-yr vs 10-yr battery replacement penalty.
Head-to-Head Parity (Risk ¿ 4%)	£12,753 (-26.8%)	£12,765 (-26.8%)	Dominated	Must offset higher 30-year replacement frequency across 1,103 implanted patients.
Head-to-Head Parity (Observed)	£12,620 (-27.6%)	£12,631 (-27.5%)	Dominated	Must offset replacement burden under unguided historic selection.
Practice Transition (S-ICD ¿ 6% vs. TV-ICD Obs)	£22,230 (+27.5%)	£25,469 (+46.1%)	£10,198 / QALY	Risk ¿ 6% clinical efficiency (avoiding low-risk implants) cross-subsidizes S-ICD hardware premium.